



Care that gets documented.
Revenue that gets protected.

EVIDENCE-FIRST DOCUMENTATION FOR AMBULATORY CARE

Revenue-assurance infrastructure for German ambulatory care

THE PROBLEM

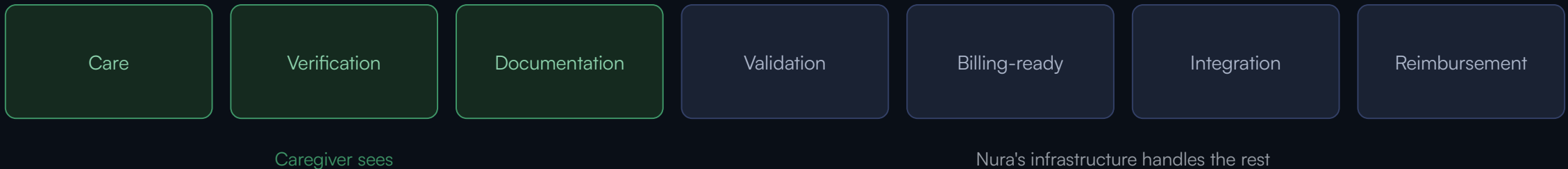
The care happened. *The paperwork failed.*

- 01 Care delivered
- 02 Documentation
- 03 Small form error
- 04 Claim problem
- 05 Administrative work
- 06 **Cashflow impact**

Ambulatory care providers in Germany run on thin margins. A missing signature, an illegible abbreviation, a wrong date turns a completed, legitimate service into a rejected claim — costing thousands of euros per provider, per month, in delayed or lost cash.

Earn trust before billing.

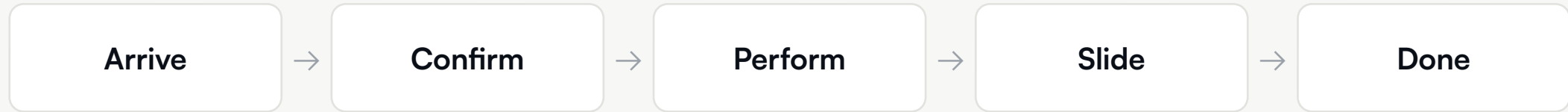
Nura turns care into a verifiable evidence chain — before a claim moves forward.



A service should not silently fall out of the chain because one required field was missed. The fix isn't more paperwork — it's making the chain itself impossible to silently break.

THE SOLUTION

Five taps. No menus, no forms.



Underneath: multi-signal visit verification, a versioned rules engine, and a billing firewall that checks every service before it's allowed to become a claim.

CLAIM VALIDATION

Patient ✓

Service ✓

Date & time ✓

Required documentation ✓

Confirmation ✓

✓ All checks passed

DIFFERENTIATION

Nura doesn't automate risk. It automates certainty.

Verify

Never trust a single signal.

Low-confidence visit matches
escalate to the caregiver instead of
guessing.

Validate

**Never send incomplete billing
data.**

The billing firewall blocks a claim
before it's incomplete, not after.

Record

**Never silently overwrite an
audit trail.**

Corrections create a new event —
the original gap stays visible.

Escalate

**Never decide automatically
under uncertainty.**

Sync conflicts lock billing and
route to a supervisor, not auto-
resolve.

BUSINESS MODEL

One managed digital work station.

HARDWARE-AS-A-SERVICE

€129

per device, per month

- Hardware + device management
- Nura software + updates
- Security, remote lock/wipe
- Offline sync + audit trail
- Support & SLA

Not "an app that costs €129."

~15,300

ambulatory care providers in Germany

420,000+

caregivers as the target market

Market-size figures are from the original concept memo; not independently re-verified in this deck.

PROOF, NOT PROMISES

Start small. Prove the economics. Scale.

01	02	03	04
Baseline	Deployment	Measurement	Scale
Measure current documentation-related rejection rate.	Introduce Nura into daily caregiver workflows.	Track rejection rate, correction time, billing readiness.	Expand only once evidence is measurable.

The KPI that matters: documentation-related billing rejection rate — measured before and after deployment, not asserted upfront.

We are not claiming 0% rejection across every cause, and pilot validation is single-site — a case study, not market-wide proof yet.

What we're not claiming today.

Not 0% billing rejection.

Only documentation/form-related rejections trend toward zero — Nura can't control every payer-side reason.

Not fraud prevention.

Nura creates an evidence chain, not mathematical proof of care.

Not a TI/KIM replacement.

Nura is integration-ready, not a replacement for regulated infrastructure.

Not market-wide proof.

Pilot validation is single-site — a case study, not proof the model generalizes yet.

THE TEAM

The founders behind Nura.



Rafael

Co-Founder & CSO

Louis

Co-Founder & COO

Ivan

Co-Founder & CTO

Oscar

Founder & CEO



**Nura protects the revenue between
care delivered and cash received.**



Scan to try the prototype